

CLINICAL DISCHARGE SUMMARY

SAMPLE / SYNTHETIC DOCUMENT FOR TESTING ONLY

Patient Name	Nisha Thomas (Synthetic)
Age / Sex	64 years / Female
Medical Record No.	TEST-64027
Date of Admission	25 August 2026
Date of Discharge	31 August 2026
Attending Physician	Dr. Rahul Iyer

1. Chief Complaint

The patient presented with a three-day history of increasing lower abdominal discomfort, nausea, reduced appetite, and intermittent low-grade fever. She reported urinary frequency but denied vomiting, chest pain, or shortness of breath.

2. History of Present Illness

Symptoms began approximately three days before admission. Abdominal discomfort was initially mild but became more persistent over the following 24 hours. The patient reported burning during urination and generalized weakness. There was no reported recent travel or known sick contact.

3. Past Medical History

History of hypothyroidism for approximately six years, controlled with medication. History of osteoarthritis of both knees. No known chronic kidney disease or previous abdominal surgery was documented.

4. Vital Signs on Admission

Temperature: 38.1 °C. Blood pressure: 142/84 mmHg. Heart rate: 98 beats/min. Respiratory rate: 18 breaths/min. Oxygen saturation: 96% on room air.

5. Physical Examination

Patient was alert and oriented and appeared mildly uncomfortable. Mild suprapubic tenderness was present without guarding or rebound tenderness. Heart sounds were regular. Lungs were clear bilaterally. No peripheral edema was noted.

6. Investigations

Complete blood count showed WBC 12,400/ μ L with neutrophil predominance and hemoglobin 12.9 g/dL. C-reactive protein was elevated at 54 mg/L. Serum creatinine was 0.8 mg/dL. Urinalysis showed leukocytes and positive nitrites. Urine culture grew a common urinary pathogen sensitive to the prescribed antibiotic. Abdominal ultrasound showed no hydronephrosis or obstructing urinary calculus.

7. Hospital Course

The patient was admitted for treatment of a suspected urinary tract infection with systemic inflammatory features. She received intravenous fluids, antibiotics, antipyretics, and monitoring of vital signs and urine output. Fever and urinary symptoms gradually improved over the next several days. Renal function remained stable, and no surgical or intensive-care intervention was required. She was transitioned to oral antibiotics before discharge.

8. Final Diagnoses

- Complicated urinary tract infection, clinically improved.
- Acute febrile illness secondary to urinary infection, resolved.
- Hypothyroidism.
- Osteoarthritis.

9. Medications at Discharge

Cefixime 200 mg orally twice daily for 5 additional days.

Paracetamol 500 mg orally every 6 hours as needed for fever or discomfort.

Levothyroxine 50 mcg orally once daily before breakfast.

Patient was instructed to complete the antibiotic course unless advised otherwise by the treating clinician.

10. Discharge Condition

Patient was afebrile, hemodynamically stable, tolerating oral intake, and able to ambulate independently. Abdominal discomfort had substantially improved, and there were no new respiratory or neurological symptoms.

11. Follow-Up and Instructions

Follow up with the primary-care physician within one week. Adequate oral fluid intake and medication adherence were advised. Return urgently for recurrent high fever, worsening abdominal or flank pain, persistent vomiting, confusion, fainting, or markedly reduced urine output.

12. Clinical Notes / Evidence

The diagnosis was supported by the patient's urinary symptoms, inflammatory markers, urinalysis findings, and urine culture. Imaging did not demonstrate urinary obstruction. The patient showed clinical improvement with antimicrobial and supportive treatment during hospitalization.

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